

--SUMMARY--

Decision No. 28/24

17-Jan-2024

R.Horne - M.Moreau - C.Salama

- Tear (ligament) (knee)

No Summary Available

12 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w1024n]
- MEDICAL DISCUSSION PAPERS CONSIDERED: Knee Injury and Disability
- TRIBUNAL DECISIONS CONSIDERED: Decision No. 2062/01R, 2005 ONWSIAT 1013 refd to; Decision No. 3503/18IR, 2020 ONWSIAT 1605 refd to

Style of Cause:

Neutral Citation: 2024 ONWSIAT 92



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 28/24

BEFORE:

R.P. Horne: Vice-Chair
M.P. Moreau: Member Representative of Employers
C. Salama: Member Representative of Workers

HEARING:

January 8, 2024 at Toronto
Oral by Videoconference

DATE OF DECISION:

January 17, 2024

NEUTRAL CITATION:

2024 ONWSIAT 92

DECISIONS UNDER APPEAL:

WSIB Appeals Resolution Officer (ARO), T. Lim dated
June 15, 2021 and the decision by ARO, M. Houghton dated
January 16, 2023

APPEARANCES:**For the worker:**

M. Farago, Lawyer

For the employer:

M. Woo, Paralegal

Interpreter:

Not applicable

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REASONS

(i) Introduction

[1] Both the worker and the employer object to separate decisions of the Appeals Service Division of the Workplace Safety and Insurance Board (WSIB or the Board). The employer objects to the June 15, 2021 decision by Appeals Resolution Officer (ARO) T. Lim that granted initial entitlement to a left knee sprain occurring on September 24, 2019. The worker objects to ARO, M. Houghton's decision dated January 16, 2023 that concluded the worker's diagnosed posterior cruciate ligament (PCL) tear of the left knee did not arise from the September 24, 2019 work accident.

[2] ARO Lim conducted an oral hearing whereas ARO Houghton did not but relied on written submissions from the parties.

(ii) Issues

[3] The issues under appeal are as follows:

1. The employer is requesting initial entitlement for a left knee strain be rescinded. (ARO decision June 15, 2021) (Withdrawn)
2. The worker is seeking entitlement for a left knee posterior cruciate ligament (PCL) tear. (ARO decision January 16, 2023)

[4] In the hearing, Mr. Woo the employer's representative requested that the issue of initial entitlement be withdrawn. The employer was no longer disputing the happening of the accident of September 24, 2019 but continued to support ARO Haughton's decision that the PCL tear was not caused by the September 24, 2019 work accident. Mr. Woo acknowledged that any attempt to re-register the appeal specific to initial entitlement would be subject to statutory time limit restrictions. The Panel accepted the withdrawal and as a result the hearing was limited to whether or not the PCL tear was a consequence of the September 24, 2019 work accident.

(iii) Background

[5] This now 44-year-old construction supervisor slipped and fell, landing on his left knee on September 24, 2019. The exact mechanics of the accident is described as the worker entering a room through a sliding door and tripped on the threshold (bottom sliding mechanism) and fell forward landing on his bended left knee. The worker indicates there was a witness and that he reported the injury to the site supervisor. He also saw his family doctor on the day of the accident. He was however able to continue working due to the supervisory nature of his job. The employer disputes that the worker reported the injury on the day of the accident.

[6] The worker reported to his employer in mid-February 2020 that he was unable to continue working due to his September 24, 2019 left knee injury. Following an MRI occurring on February 21, 2020 he was diagnosed with a PCL tear and surgery was being contemplated.

[7] The worker indicated that on or about December 25, 2019 he experienced increased left knee pain when his young daughter was sitting on his knee and again on the same day while going up some stairs. In addition, he had an acute onset of left knee pain while kneeling on February 11, 2020, necessitating a trip to a hospital emergency room. The Board initially denied

entitlement to the September 24, 2019 work accident largely due to the delay in reporting and the intervening events. On appeal, ARO Lim accepted proof of accident noting the worker had sought medical treatment the same day but denied any loss of earnings benefits beyond February 2019 due to the presence of intervening events that were more than likely the reason for the ongoing left knee condition. ARO Haughton has subsequently ruled the PCL tear was not caused by the September 24, 2019 work accident.

(iv) Law and policy

[8] *The Workplace Safety and Insurance Act, 1997* (the WSIA) is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[9] An “accident” is defined in section 2(1) to include:

- (a) a wilful and intentional act, not being the act of the worker,
- (b) a chance event occasioned by a physical or natural cause, and
- (c) disablement arising out of and in the course of employment.

[10] General entitlement to benefits is governed by section 13:

13(1) A worker who sustains a personal injury by accident arising out of and in the course of his or her employment is entitled to benefits under the insurance plan.

(2) If the accident arises out of the worker’s employment, it is presumed to have occurred in the course of the employment unless the contrary is shown. If it occurs in the course of the worker’s employment, it is presumed to have arisen out of the employment unless the contrary is shown.

[11] The statutory presumption set out in section 13(2) does not apply to an injury by disablement. See, for example, *Decision Nos. 268, 1987 CanLII 1992 (ON WSIAT)* and *42/89, 1989 CanLII 2559 (ON WSIAT)*.

[12] Tribunal jurisprudence applies the test of significant contribution to questions of causation. A significant contributing factor is one of considerable effect or importance. It need not be the sole contributing factor. See, for example, *Decision No. 280, 1987 CanLII 1996 (ON WSIAT)*.

[13] The standard of proof in workers’ compensation proceedings is the balance of probabilities.

[14] Tribunal jurisprudence has also established that it is not required that the workplace accident be the sole cause of the worker’s condition. It need only be determined that the workplace injury or activity was a “significant contributing factor” to the worker’s permanent condition. If both non-compensable and compensable factors are found to be significant contributing factors in the development of a disability, entitlement may still be allowed in such circumstances. As the Vice-Chair stated in *Decision No. 2062/01R, 2005 ONWSIAT 1013*, to establish entitlement, it is not necessary to show that the compensable injury was the sole contributing factor or even the predominant contributor. Rather, the compensable injury need only be a cause of the disability, providing that it makes more than a de minimus contribution.

[15] Pursuant to subsection 124(2) of the WSIA, the benefit of the doubt is resolved in favour of the claimant where it is impracticable to decide an issue because the evidence for and against the issue is approximately equal in weight.

[16] Pursuant to section 126 of the WSIA, the Board stated that policy packages #241 and #300, revision #9, would apply to the subject matter of this appeal.

[17] We have considered these policies as necessary in deciding the issues in this appeal.

(v) Documentary and medical evidence

[18] The Panel has reviewed the entirety of the Case Record and have found the following documents particularly useful in our understanding of the case.

[19] ARO Houghton explained her rationale for denying the PCL tear as follows in her January 16, 2023 decision.

I find the contemporaneous medical evidence does not support a PCL tear. There was no indication of a PCL tear during the initial assessment of the worker's knee on September 24, 2019. The findings provided from this assessment are not consistent with a significant injury to the left knee or with a PCL tear. I note the February 29, 2020 report from an orthopaedic specialist indicated the worker had a work-related injury when they tripped and fell on their left knee in September 2019, which was treated and ultimately improved. The specialist noted that ultimately the worker reinjured their knee, twisting it on some stairs around Christmas time.

I find there is insufficient evidence to support the re-emergence of left knee symptoms the worker experienced in December 2019 was casually related to the September 24, 2019 work-related injury. I note the prior ARO determined there lack of evidence of continuity of symptoms from the date of accident until the next clinical documentation of a left knee problem at the end of December 2019. The ARO found the three (3) month break in evidence was significant and they were unable to reconcile it. I note that while additional medical information was received to the claim file, this break has not been reconciled.

I find there is a break in the chain of causation as the worker sustained a subsequent non-occupational injury to the left knee in December 2019, after which a diagnosis of a PCL tear was provided. I find there is no proof of a work-related injury to the left PCL, specifically a left knee PCL tear, as a result of the September 24, 2019, workplace accident.

[20] The worker in his verbal statement to the Board on March 3, 2020 described the nature of the accident:

My foot (unsure of which foot) got caught on the ledge of a sliding door as I was walking from the balcony into the suite. I stumbled and ended up falling and landing on my left knee.

- Walking from balcony into the suite
- Foot caught ledge of sliding door
- Stumbled and fell down landing on left knee
- There was knee pain while I stumbled and pain when I landed

[21] The Emergency Room report of February 11, 2020 outlined the worker's presentation on that day:

Subjective Assessment: Pt had a minor fall/hyperextension few months ago and since then has been wearing Left knee brace. Last night pt was kneeling down and pain started to left knee, pt states "feel it moving to side". No other injury.

[22] Dr. F. Mastrogiacomo, an orthopedic specialist, examined the worker's knee and had the following comments in his February 19, 2020 consultation report:

[The Worker] is a 40-year-old gentleman, who is a labourer by trade, unfortunately sustained a work-related injury tripping and falling onto his left knee back in September.

He fell directly onto the knee. I believe, by description, he slipped or tripped over a sliding door ledge, possibly at a worksite, he was not totally clear on the nature of the injury. Nonetheless, it appears as though it was work related. He ultimately went to see his family physician. Medications were prescribed and he ultimately did improve. He was using a neoprene sleeve or similar type of sleeve for general pain control and to improve his sensation of stability in the knee. Ultimately, the patient reinjured the knee, twisting it on some stairs at around Christmastime. This was not work related based on his description. We then eventually locks follow up. He complained of obvious locking of the knee. He stated that he was able to unlock the knee and his knee had ultimately recovered.

Unfortunately, just last week, his pain increased again and he developed an inability to fully extend the knee and now he has been left with pain in the knee. He ultimately presented to our emergency Department on February 11th because of ongoing pain in the knee. He was seen by the Emergency Department and ultimately referred to the Fracture Clinic.

[23] The worker's representative, Mr. M. Farago, previously submitted several medical articles and excerpts from medical journals in support of his case. Portions of these articles are reproduced below.

[24] From Web MD "What Is a PCL Injury? What Does It Feel Like?":

What Causes It?

Many knee injuries are sports related. But a PCL injury is more likely to happen during trauma like a car accident.

There are a number of ways your PCL can be damaged:

Something forces your knee backward very quickly -- a car crash, for example.

Your shin slams very hard into something, like your car's dashboard.

Your knee twists or over-extends in a sudden movement.

You fall down or get tackled, landing on your bent knee as your foot is pointing downward. This can happen while playing sports like football or soccer.

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What Are the Symptoms?

Most people who damage their PCL feel pain and have swelling. Your knee may feel stiff and sore. It might also feel unstable. Or the joint may feel loose, as though it can't support your weight.

You may have trouble walking.

If you only have mild damage to the PCL and no other parts of your knee, you may not notice pain, swelling, or other problems at first. These symptoms may appear over time.

What is a posterior cruciate ligament (PCL) injury?

A PCL injury is a sprain or tear of the posterior cruciate ligament (PCL). The PCL is a band of tissue that crosses inside the centre of the knee joint. It connects your thigh bone to the bone of your lower leg. The PCL keeps your knee stable when it moves forward or backward.

A direct blow to the knee can injure your PCL. For example, the PCL can be injured in a car crash when your bent knee hits the dashboard. You can also hurt your PCL during sports, such as football, soccer, or skiing. Or you can hurt it while doing other activities if you fall on your bent knee with your foot or toes bent downward or if the front of your knee is hit.

A PCL injury can also happen if you stretch or straighten your knee beyond its normal limits (hyperextend the knee).

What are the symptoms?

An injury to your PCL may cause:

Swelling, pain, tenderness, and stiffness. Several hours after the injury, your pain may get worse. And it might be harder to move your knee.

Bruising.

An unstable feeling, like the knee may give out.

[25] The Cleveland Clinic “*Posterior Cruciate Ligament (PCL) Injury: Symptoms & Treatment*” outlines the signs and symptoms of a PCL injury:

PCL injuries usually occur with severe knee trauma. You may develop a problem with your PCL if you:

- Fall onto a bent knee.
- Are hit hard on the front of your knee (think dashboard in a motor vehicle accident).
- Bend your knee too far backward. Dislocate your knee.
- Land improperly after a jump.

What are the symptoms of a posterior cruciate ligament injury?

People with PCL injuries may experience a wide range of symptoms, including:

- Pain that worsens over time. Swelling and inflammation.
- A feeling of instability in the knee. Stiffness.
- Difficulty walking.
- Trouble going down the stairs.

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If your PCL injury is minor, you may heal without complications. However, if the ligament sustained severe damage, your knee could become weak and prone to re-injury unless you have surgery.

People who undergo PCL surgery generally notice improved stability and mobility following recovery. Keep in mind, however, you may still need to wear a knee brace during physical activities to protect your joint.

[26] Also, the article “sports-injuries/knee-injuries/posterior-cruciate-ligament-pcl-injuries” from the website sports-health.com by Dr. McCabe outlines the common causes of a PCL injury:

Posterior cruciate ligament (PCL) injuries can go undiagnosed because they are generally less painful and do not cause as much instability as other knee ligament injuries, such as an anterior cruciate ligament (ACL) tear or medial collateral ligament (MCL) tear. In fact, it is possible for some PCL tears to go undiagnosed for weeks or even years.

The PCL of the knee can be injured during contact sports, such as football, soccer and skiing, as well as during a car accident. PCL injuries commonly occur in the following instances:

A direct blow to the knee, such as when a soccer player falls onto a bent knee Over-twisting or hyperextending the knee, such as while skiing or quickly changing direction on one foot

Landing on the front of the shinbone, such as when a dancer is coming down from a leap

A simple misstep that results in falling on a bent knee

A bent knee hitting the dashboard during a car accident

While generally less painful than other knee ligament tears, a PCL tear can still cause pain, swelling, and decreased range of motion.

[27] Finally, the article “Rupture of Posterior Cruciate Ligament: Diagnosis and Treatment Principles” appearing in *Knee Surg. Relat. Res.* 2011;23(3):135-141, notes isolated PCL injuries can be difficult to diagnose:

Posterior cruciate ligament (PCL) injuries associated with multiple ligament injuries can be easily diagnosed, but isolated PCL tears are less symptomatic, very difficult to diagnose, and frequently misdiagnosed. If a detailed investigation of the history of illness suggests a PCL injury, careful physical examinations including the posterior drawer test, dial test, varus and valgus test should be done especially if the patient complains of severe posterior knee pain in >90 degrees.

[28] The Medical Liaison Office of the Tribunal included the Medical Discussion Paper “Knee Injury and Disability” authored by Dr. J. Cameron, 2013 as revised by Dr. R. Litchfield in 2022. This paper does not necessarily represent the views of the Tribunal and its content is not binding on the Tribunal in any particular case. It is always open to the parties to an appeal to rely on or distinguish any medical information, and to supplement or to challenge it with alternative evidence. The Medical Discussion Paper Knee injury and disability however does discuss the importance of knowing the mechanism of injury and makes a comment concerning PCL tears. It states the following:

What is the Mechanism of Injury?

The mechanism of injury will often be useful in determining the severity of the injury and the possible structures damaged.

Direct Blow: (fall onto a flexed knee), could result in fractures of the knee cap or damage to the patella or trochlea (the groove the patella articulates with) cartilage, or bruising (contusion) to the soft tissues in front of the knee. The posterior cruciate ligament (PCL) tear may also occur from this mechanism, albeit less commonly.

(vi) Analysis

[29] The appeal is allowed.

[30] Notwithstanding that the employer has withdrawn their objection to the allowance of initial entitlement the Panel has found it necessary to examine the mechanics of the work accident. We do so in order to accurately assess the compatibility between the work accident of September 24, 2019 and the diagnosed PCL tear of the left knee.

(a) Mechanics of the accident

[31] There are minor discrepancies in how the accident was reported. The worker testified under oath in the WSIB hearing occurring on May 14, 2021 and as recorded in the June 15, 2021 ARO decision that “the work accident involved his tripping on a balcony ledge when re-entering a condominium unit from the balcony. He fell onto his bent left knee.” In the hearing before us on January 8, 2024 the worker went into greater detail. He advised he was out on the balcony of a condominium construction project having a cigarette and talking to a co-worker. On re-entering the condominium, he stumbled over the patio door tracking mechanism. He stumbled forward a few steps trying to regain his balance and then fell forward with his arms outstretched and landed on his left knee which was bent almost up to his chest area due to his stumbling motion.

[32] The worker testified he completed a “Near Miss /Incident Report” that day at the general contractor’s office. That report does not mention a fall onto the knee. It states “walking from balcony into unit, clipped edge with foot causing pain in the knee.” The worker also on September 24, 2019 saw his family doctor Dr. W. Khan. Dr. Khan’s chart note for the same date described the accident as follows. “Lt knee injured today, feels tense, painful injury at work, tripped caught his knee on falling”. This description does not implicitly indicate the worker fell onto his knee but it seems to be implied noting the notation of “falling”. We note as well the witness statement dated October 23, 2020 that indicates “as [the worker] was stepping back into the unit, he mis-stepped and his knee buckled. He complained about his knee that day and for a couple of days after that. I also saw him limping.” This statement as well does not confirm a specific fall onto the knee.

[33] Both the employer and the worker submitted accident reports to the WSIB. The employer’s report of injury/Disease dated March 4, 2020 notes the worker reported on February 20, 2020 that on September 24, 2019 “his boot was caught on the ledge of the sliding door causing him to fall into the unit”. The same document records a left knee injury. The “Worker’s Report of Injury/Disease” is dated February 20, 2020 and it described the September 24, 2019 accident as “foot got caught on ledge of sliding door when walking from the balcony into the suite, stumbled in pain and landed on knee”. The worker saw the specialist, Dr. Mastrogiacomio, on February 19, 2020. In this report the specialist also described the mechanics of the accident. He confirmed it involved falling directly on the left knee. “[The worker] unfortunately sustained a work-related injury tripping and falling onto his left knee back in September. He fell directly onto the knee.”

[34] As stated above there are minor variances in the accident histories as described by various people at various times. They are in the main consistent. The accident occurred at work and involved a trip or stumble over the patio door threshold. Some of the histories can be read that the worker slipped and twisted and/or or caught the knee. The most contemporaneous report of the accident arguably is the near miss report completed by the worker on the day of the accident that merely stated he clipped the edge and felt pain in the knee. However, on the same day the family doctor appears to confirm the worker actually fell onto and injured the knee. Both the worker’s and the employer’s report to the WSIB state there was a fall. Both testimonies of the worker at hearings before the WSIB and the WSIAT, two years apart, confirm a trip, resulting in a fall directly onto a bent knee. The specialist, Dr. Mastrogiacomio also supports a fall directly on the knee. On balance, the Panel finds the preponderance of the evidence supports a slip and fall onto the front of the knee. We further accept the worker’s testimony that the knee was bent at the time.

(b) Left knee symptoms between September 24th and December 25, 2019

[35] The employer's representative argued that the evidence is not supportive of the September 24, 2019 work accident as being responsible for the left knee PCL tear. He noted the lack of reporting at the time of the accident, the fact the worker was fully capable of performing his regular duties from September 2019 to February 2020 without complaint or accommodation and the documented incidents in December 2019 that could reasonably be an alternate cause of the PCL tear. We reject that view due to the evidence of ongoing problems with the knee prior to the December 25th event and due to the fact the mechanics of the accident as described above are more compatible with a PCL tear than the incidents described subsequently.

[36] We find that the worker had symptoms of instability in the left knee following his September 24, 2019 work accident. The worker testified that following the September 24, 2019 accident his knee felt unstable. He had difficulty going up stairs. He attempted to limit the stairs by taking the freight elevator if he had to travel up or down multiple stories. He also purchased an off the shelf knee brace which the worker wore under his clothes. He denied taking any significant medication but would occasionally have to massage his knee for relief. He indicated he could achieve some relief by pushing on the back of his knee, forward. He indicates he did report the accident to his immediate supervisor the day after the accident but admitted it was in the midst of a heated argument over the progress of the job. He described his supervisor as being difficult and he was reluctant to complain to him about his knee although others apparently knew of his ongoing pain. He had hoped the knee pain would go away and was under the impression that it was best not to report an injury to the WSIB unless it was serious.

[37] The Panel does not accept the view that the September 24, 2019 knee injury was minor and the worker fully recovered within a short period of time and that further episodes of knee pain were unrelated. We do so in part due to the worker's testimony outlined above. We also note Dr. Khan's chart note of December 31, 2019 which indicates the worker had been using a brace on the knee for several weeks now. This confirms to the Panel's satisfaction that the worker was having left knee pain sufficient to use a brace between September 2019 and December 25, 2019. This view is further support by the comment by both the emergency room physician and Dr. Mastrogiacomo in his consultation report of February 19, 2020. The ER physician referenced an injury a few months ago and that since that time the worker had been wearing a knee brace. This conforms with Dr. Khan's report that even before Christmas the worker was using a brace. The specialist's report of February 19, 2020 also noted the September fall onto the knee which the doctor indicated improved but noted the worker continued to use a brace until the re-injury at Christmas and further flare in February 2020. Taken together we are satisfied the worker remained symptomatic with his left knee from September 2019 to late December 2019, had instability in the knee and used a knee brace for symptomatic control.

(c) Compatibility between the September 24, 2019 accident and the diagnosis of PCL tear

[38] We rely on the medical reporting from the worker's treating practitioners and that of medical articles submitted by the worker representative, Mr. M. Farago as well as the Tribunal's own Medical Discussion Paper "Knee Injury and Disability".

- [39] We find it is more likely than not the worker's PCL tear resulted from his September 24, 2019 work injury. We do so given the evidence outlined above that the worker had ongoing symptoms beyond September 24, 2019 through to his layoff from work in February 2020 and due to the view the intervening events described on December 25, 2019 and February 11, 2020 would not be compatible with a PCL tear.
- [40] We start with a review of the Tribunal Medical Discussion Paper. It notes that PCL tears are less common than ACL tears and specifically mentions that they can occur due to a direct blow to the knee such as a fall onto a flexed knee. We note this is the exact mechanism described by the worker. The medical article submitted from the Cleveland Clinic entitled *Posterior Cruciate Ligament (PCL) Injury: Symptoms & Treatment* also describes a fall onto a bent knee as a primary cause of a PCL tear as does the Web MD article "*What Is a PCL Injury? What Does It Feel Like?*". In addition, we note the medical article from the journal *Knee Surg Relat Res* 2011;23(3):135-141, <http://dx.doi.org/10.5792/ksrr.2011.23.3.135> ISSN 2234-0726 ISSN 2234-2451, "Rupture of Posterior Cruciate Ligament: Diagnosis and Treatment Principles" which indicate that PCL injuries are difficult to diagnosis, particularly if other ligaments are not involved. The worker's MRI of February 21, 2020 fails to show any other significant left knee pathology so would fit this scenario. The article goes on to note that the symptoms may be mild and worsen over time, which is the case in the matter before us. All of the submitted articles relate knee instability as being the primary symptom of a PCL tear. We accept that this symptom is also the primary problem this worker experienced. Again, we are satisfied due to the use of the brace that the left knee instability was present prior to the recorded events on December 25 and February 11, 2020.
- [41] The worker indicated that on December 25 while going up a few steps shortly after his daughter had been sitting on his lap, the knee "locked" and the same thing occurred on February 11, 2020 when he was kneeling. Rather than these subsequent events being responsible for the PCL tear we find it more likely than not the intervening events represent the manifestation of a symptom (locking) that is characteristic of an already present PCL tear. For all of the reasons noted above we find there is compatibility between the September 24, 2019 work accident and the later diagnosed PCL tear.
- [42] Mr. Woo, the employer's representative, referred to the family doctor's clinical chart note of September 24, 2019 and the fact the doctor appears to have completed a thorough medical examination of the knee. The chart note records that the worker's left knee showed "no erythema, no induration, no effusion, no warmth, no obvious deformities, no asymmetry, skin intact, no rash, no bruising". The doctor goes on to note the worker was not tender on palpation. Mr. Woo relies on this note to suggest in the absence of an obvious deformity a PCL tear was not present. We however note the submitted medical literature that notes isolated PCL tears are difficult to diagnosis and also note that Dr. Mastrogiacomo the orthopedic surgeon, who presumably has more expertise in diagnosing knee injuries also initially, mis-diagnosed the knee injury, initially believing it to be an ACL tear. We therefore put little weight to the fact the tear was not diagnosed immediately by Dr. Khan.
- [43] We acknowledge the testimony from the employer's witness, the Health and Safety Manager. She confirmed that accidents are reported initially to a supervisor and then to her. She was unaware of the worker's left knee injury until February 20, 2020. She confirmed the worker made no complaints and completed all of his regular duties which included stair climbing and significant walking from September 2019 to February 2020. She testified that she visited the site

a few times during this period and did not notice the worker favoring the knee or limping. To the best of her knowledge the worker was laid off on February 14, 2020 but conceded she is not involved in that process.

[44] It is not disputed that the worker continued with his regular duties. His own testimony is that he tried to limit the amount of physical work and used the freight elevator to travel between floors when he could. We also note the medical article, quoted above, that indicates individuals with a mild PCL tear can carry on much as before, however symptoms tend to worsen over time. This to the Panel is an adequate explanation as to why there is a lack of complaint or need for overt accommodation.

(d) Entitlement to loss of earnings (LOE) benefits

[45] We make no finding on the worker's entitlement to loss of earnings (LOE) benefits. We do so in part because both AROs in their decisions were silent on LOE entitlement. LOE entitlement however could be seen as a sequential issue or an issue the ARO would have ruled on if they had concluded differently. Tribunal case law holds that the Tribunal has jurisdiction over sequential issues. See Tribunal *Decision No. 3503/18IR*, 2020 ONWSIAT 1605, for a more detailed discussion on adding sequential issues. We however decline to take on jurisdiction of LOE entitlement due to a lack of credible information within the Case Record. The worker testified he left work on February 11, 2020 due to his knee symptoms. The employer indicates he was laid off due to shortage of work on February 14, 2020. These opposite positions were not reconciled in the hearing. The worker's specialist Dr. Mastrogiacomo indicates in his consultation note of March 4, 2020 that the worker would be disabled from labouring type work. That is the last medical document on the Case Record. The worker testified he went on short term disability through his union from February 2020 to September 2020 when he secured a new job as a construction framer (Metal). When asked why he did not return to a construction foreman position he replied it was because he did not enjoy the job as it came with too much pressure. Given the above there is a lack of medical reporting substantiating ongoing total disability beyond March 2020. There are no records from the short-term disability carrier for example and limited information regarding the worker's efforts to mitigate his wage loss during the period he was off work. For all of the above reasons the Panel remits back to the Board the determination of entitlement to LOE benefits (if any) beyond February 11, 2020. The worker retains the usual rights of appeal from any decisions flowing from this direction.

DISPOSITION

[46] The appeal is allowed as follows:

1. It is more likely than not the worker's left knee PCL tear arose as a consequence of his September 24, 2019 work-related accident.
2. The issue of entitlement to LOE benefits beyond February 11, 2020 is remitted back to the Board for determination.

[47] The nature and duration of benefits flowing from this decision will be returned to the WSIB for further adjudication, subject to the usual rights of appeal.

DATED: January 17, 2024

SIGNED: R.P. Horne, M.P. Moreau, C. Salama